

Daily Update — Dad's care, 2026-05-19

AT A GLANCE

- **First IV ketamine session at Royal Oak yesterday (5/18).** Difficult in the chair — shaking, crying out. Afterward Dad was more coherent than Mom has seen him in a long time; he himself said he didn't feel better.
- **Dad and Julane will continue weekly sessions through the gap before the June 8 ketamine restart.** This is a meaningful new piece of continuity — integration and behavioral work happening alongside any medication decisions rather than being held until the next infusion.
- **Royal Oak: off the table. CA: off the table per Dad.**
- **Ann Arbor clinic start: June 8** (revised from tentatively early June). Chilton pushed it back to allow the Cymbalta → Lexapro switch to complete first.
- **Four OpenEvidence consultations have come back since this morning** — on SSRI vs SNRI as ketamine background (#10), divalproex in this profile (#11), RBD episode-frequency trajectory (#12), lemborexant onset speed (#13), and bupropion augmentation in this activation-intolerance profile (#14). They land directionally. Summarized below.
- **Headline reads:** (a) the published evidence on the Cymbalta → Lexapro switch is mixed — the OE response lands against it on Del Casale 2025 + escitalopram-already-failed grounds, but Dad's decades-long prior-responder history on Lexapro is real prior-responder data the OE didn't have; the load-bearing question is now **sequencing** rather than direction (do the cross-titration *after* the first 2–3 ketamine sessions, not before); (b) divalproex has no defensible indication and serious drug-induced parkinsonism risk in Dad's profile; (c) the decreased RBD episode frequency does *not* relax the conservative medication posture and may reflect confounders (duloxetine REM suppression, melatonin, separate sleeping); (d) lemborexant is night-1 effective and a seamless substitute for doxepin if doxepin needs to come off; (e) bupropion augmentation is a defensible alternative to divalproex if augmentation beyond ketamine is desired, with fall risk as the load-bearing caveat.
- **Dad's emotional state:** dejected; explicitly asked the family to maintain genuine (not false) hopefulness.

WHAT WE KNOW

- **5/18 session at Royal Oak:** shaking, crying out, prolonged distress. "Going to Africa." No provider in the room.
- **Post-session functional state on 5/18:** "more coherent than Mom has seen him in ages." Dad said he didn't feel better in the chair. The session experience and the response are separate variables.
- **Dad's processed experience after Julane (5/19):** didn't feel attached to anyone; runaway train; not in a position to walk through the door ketamine was opening. February felt supported (family around him); 5/18 felt alone.
- **Julane continuity:** Dad will keep meeting with Julane weekly through the gap to June 8. This is the strongest practical lever on the "felt alone" / set-and-setting dimension that distinguished 5/18 from February.
- **February 2026 ketamine course was IV** and produced a dramatic ~4-week response on a duloxetine + brexpiprazole background. Dad is specifically an IV responder. Later lozenge use was good but less transformative.
- **Dad did not take the proposed quetiapine.** See [handout #18](#).
- **A2 ketamine start: June 8.** Chilton's stated rationale: allow Cymbalta → Lexapro switch to complete first.
- **Current sleep stack:** doxepin 6 mg HS, trazodone 50 mg PRN HS, melatonin 3 mg HS. See [handout #13](#) and [handout #19](#).
- **Current antidepressant:** duloxetine 60 mg, on board since 11/2025 (~6.5 months). The documented Feb 2026 responder background.

- **Bupropion has never been on board.** Considered and held in clinical brief; family lit review #11 (4/20) explicitly excluded it on adrenergic-intolerance grounds — an exclusion the OE bupropion response (see below) explicitly judged "too broad."
- **RBD episode frequency trajectory:** decreased over the past year; most recent ~February 2026; now roughly every few months; no dream recall recently. Multiple confounders make this uninterpretable as a stabilization signal (see OE summary below).
- **Sleep pattern detail:** one nocturia trip per night, no dream recall, wakes unrested, cannot nap.
- **Caffeine intake actual:** one cup with breakfast, nothing after. (Mom's report was accurate.)
- **Cognitive in-call observation (5/19):** Dad's memory was spotty and hesitant — didn't initially recall a call from ~1 hour earlier. Less impaired than San Francisco period.

OPENEVIDENCE RESPONSES — WHAT THEY TELL US

- **OE #10 + #15 — Should Dad switch from Cymbalta (an SNRI antidepressant) to Lexapro (an SSRI) before the June 8 ketamine restart?**
 - *The core reframe — Dad's decades-long steady-state dose vs the escalation that began the recent medication cycle.* Per the 4/21 audit, Dad's documented decades-long maintenance dose of Lexapro was **30 mg/day** — itself above the standard adult range. When the depressive episode escalated in 2025, the dose was pushed further to **40–50 mg**; that's the dose range that didn't hold and prompted the switch to Cymbalta in 9–10/2025. The OE #15 follow-up matters here. The published dose-response curve for escitalopram (Furukawa 2019 meta-analysis; SERT receptor-occupancy studies) shows a clear pharmacological ceiling at ~20 mg: above that, the drug provides no additional antidepressant benefit, only mounting side effects. So the escalation to 40–50 mg was operating in the flat region of the curve — it added side effects but couldn't add antidepressant benefit. The escalation, in retrospect, looks like the intervention that began the recent medication-churn cycle rather than a true failure of Lexapro at the dose range that historically worked for Dad. **One caveat the OE response didn't address directly:** Dad's historical steady-state of 30 mg was itself above the meta-analytic ceiling, so his individual response pattern is somewhat in the tail of the published dose-response distribution. The proposed restart cap of 5–10 mg (which the OE evidence robustly supports on geriatric and ceiling grounds) is *below* his historical effective dose, not at it. Whether 5–10 mg can do the antidepressant-background work that 30 mg historically did for him is an open question the OE response also flagged via the severity-stratified framing below.
 - *What the broader evidence says:* A large 2025 study of over 55,000 patients (Del Casale) found that people on the SNRI class (like Cymbalta) actually had better outcomes with ketamine than people on the SSRI class (like Lexapro) — meaningfully lower death rates (5.3% vs 9.1%), fewer hospitalizations, and fewer depression relapses. The biology lines up with this: ketamine works partly by activating the same brain chemistry SNRIs target, so the two may reinforce each other rather than compete.
 - *The reinstatement-failure risk.* Two 2025 systematic reviews found that 4–57% of patients (pooled rate 16.5%) fail to re-respond when previously-effective antidepressants are restarted after a gap. The 7-month gap and Dad's many prior trials put him in the unfavorable region of that risk distribution. Amsterdam's data on step-wise loss of effectiveness — roughly 20% reduction in response per additional antidepressant trial — compounds the same direction.
 - *The severity-stratified question that's the sharpest analytical point.* Dad's historical response was at *mild-to-moderate* depression on 10–20 mg. The current episode is *severe* (PHQ-9 = 24, worst in 40 years). OE #15 framed this directly: "If escitalopram 10–20 mg was adequate for mild-to-moderate depression but the current episode is severe, there is no published basis for expecting 5–10 mg to control the current episode as a standalone antidepressant." Lexapro at the historically effective dose may "work" in the sense it always did, but the relevant question is whether it provides adequate background support for the ketamine course at this severity.
 - *Two specific concerns with the switch in the next three weeks:* coming off Cymbalta typically produces a 2–6 week window of "discontinuation symptoms" (anxiety, dizziness, nausea, electric-shock sensations) that would overlap with and confound the ketamine restart; and starting Lexapro carries a 2–4 week window of possible agitation, especially in older adults with Dad's history of bad reactions to other medications. The 3-week cross-titration

timeline OE #15 explicitly judged **inadequate** — published cross-titration principles suggest a minimum of 6–8 weeks for safe duloxetine taper + escitalopram start in an elderly patient with anxiety-predominant TRD and withdrawal sensitivity.

- *One sobering general finding from OE #15.* Holper 2020 reanalysis with age as a covariate: *no antidepressant provides balanced benefit (efficacy exceeding adverse events) in patients aged 70 and over*, because adverse events exceed efficacy at all doses in this age group. This is a population-level finding rather than a verdict on Dad specifically, but it argues against investing energy in switching antidepressants when no AD switch is likely to be the load-bearing intervention.
- *The dose-cap is robustly supported if the switch proceeds.* FDA-recommended maximum for elderly is 10 mg. Geriatric pharmacology means 10 mg in a 76-year-old $\approx$ 15 mg in a younger adult (50% increased AUC). Cardiac safety: QTcF prolongation is 4.5 ms at 10 mg vs 10.7 ms at 30 mg — relevant to Dad's LAFB + bradycardia. There is no evidence-based rationale for exceeding 10 mg in this patient.
- *Family read:* The switch *direction* remains partly defensible given Dad's prior responder status at the steady-state dose range — and the ceiling-effect reframe makes the "previously failed at 30–50 mg" framing weaker than it first appeared. But the *timing* — doing the cross-titration in three weeks before the ketamine restart — is the load-bearing concern, now reinforced by OE #15's explicit finding that the timeline is inadequate. **Recommended ask to Dr. Rubin:** run the first 2–3 ketamine sessions on the existing Cymbalta background, revisit the switch at the mid-June check-in with response data in hand. If pre-ketamine timing isn't winnable, ask for a gradual cross-titration over 6–8 weeks (not 3), cap Lexapro at 10 mg with a 5 mg start, define explicit reversal triggers (akathisia, persistent agitation, new motor symptom, any fall, PHQ-9 rise $\geq$ 5), and consider delaying the ketamine restart until the new background is stable. See [handout #06](#).
- **OE #11 — Should Dad start Depakote (divalproex)?**
 - *What the evidence says:* There are no controlled trials supporting Depakote as add-on treatment for depression (as opposed to bipolar disorder) at any age. It's not recommended for this purpose in any major clinical guideline. The largest study of medication combinations for hard-to-treat depression in older adults didn't even include Depakote in the comparison — the well-evidenced options for that purpose are aripiprazole, bupropion, and lithium.
 - *The specific concern in Dad's profile:* Depakote can cause Parkinson's-like symptoms (tremor, stiffness, slowed movement). In patients with early-warning signs of an underlying Parkinson's-spectrum process like Dad (formal REM Sleep Behavior Disorder since 2017, recent gait and balance findings, longstanding constipation), there's evidence the drug can *unmask* underlying disease earlier than it would otherwise emerge. A 2023 UK study of 1,433 newly diagnosed Parkinson's patients found valproate exposure was associated with the diagnosis, and the link persisted even years after the prescriptions ended.
 - *Other concerns specific to Dad's stack:* Depakote raises doxepin (Dad's current sleep medication) blood levels by 2.2 $\times$; a small study suggests it may reduce ketamine's antidepressant effect; and starting it now would confound the upcoming U-M neurology evaluation — any new motor or cognitive finding couldn't be cleanly attributed to drug vs underlying disease.
 - *Family read:* Defer. If "do more than ketamine" pressure persists, bupropion augmentation (handout #21) is a much stronger evidence-based alternative. See [handout #01](#).
- **OE #12 — Dad's REM Sleep Behavior Disorder (RBD) episodes have decreased in frequency over the past year — does that suggest his risk has gone down, and can the conservative medication approach be relaxed?**
 - *What the evidence says:* No major study has ever validated RBD episode frequency as a predictor of how the underlying brain process is going. More concerning: in Parkinson's patients followed over time, the visible RBD episodes can actually *decrease* while the underlying sleep-study findings get worse. A decrease in episodes doesn't reliably indicate the disease is stabilizing.
 - *Dad's specific picture:* the decreased episodes likely have multiple non-disease explanations — Cymbalta itself reduces REM sleep (the stage where RBD happens); melatonin has been treating it since 2017; Dad and Mom sleep separately so milder episodes go unwitnessed; and the decreased dream recall Dad reports can, in some cases, be a sign of progression rather than improvement. Meanwhile, Dad's combined profile (age 76, 8.5 years of RBD, constipation, gait findings) places him in the highest-risk group for progression per a 2021 clinical scoring system.

- *Family read:* The conservative medication approach — avoiding agents like Depakote, lithium, and certain antipsychotics that can trigger or worsen Parkinson's-like effects — should stay in place. The decreased episode frequency isn't a green light to relax that posture.
- **OE #13 — Lemborexant (Dayvigo) for sleep — how quickly does it work? Could it substitute for doxepin if doxepin needs to come off?**
 - *Speed of onset:* Lemborexant works from the very first night. A large 2019 trial in adults aged 55+ found that 5 mg HS reduced time awake during the night by about 33 minutes compared to placebo on nights 1–2 (10 mg reduced it by 42 minutes). The benefit specifically for the second half of the night — Dad's pattern of waking too early and unable to get back to sleep — is present from night 1, with no buildup period needed.
 - *Switching from doxepin:* The transition from doxepin 6 mg to lemborexant 5 mg can be done on a single night with no expected gap in sleep coverage. Both produce first-night effects.
 - *Reversibility:* If lemborexant doesn't work or causes side effects, it can be stopped abruptly with no rebound insomnia and no withdrawal — making the trial genuinely low-risk and easy to back out of.
 - *Interactions:* No significant interaction with Cymbalta, low-dose doxepin, or melatonin at Dad's doses.
 - *The remaining caveat:* Lemborexant slightly increases REM sleep time — which in theory could give more opportunity for RBD episodes — but this hasn't been demonstrated in actual patients with RBD. Worth tracking on starting.
 - *Family read:* Defensible — either as a substitute for doxepin (if doxepin is coming off) or as an addition if doxepin stays. Julane has endorsed it. Start at 5 mg HS. See [handout #10](#).
- **OE #14 — Should adding Wellbutrin (bupropion) to Cymbalta be considered as an alternative to either switching to Lexapro or adding Depakote?**
 - *Background:* Bupropion has never been tried for Dad. The family's prior research review (April 20) ruled it out on the reasoning that Dad's history of bad reactions to multiple medications made adrenergic-acting drugs too risky. The OE response specifically tested that exclusion and concluded it was "too broad."
 - *Why the prior exclusion was too broad:* Bupropion has essentially no effect on serotonin (so the "high-serotonergic mechanisms" concern doesn't apply). Mirtazapine — the drug whose agitation triggered the activation concern — works through a fundamentally different mechanism than bupropion does, even though both touch the body's stress-response chemistry. Mirtazapine causes a *burst* of stress chemicals into the brain by blocking a feedback receptor, while bupropion produces a gentler, more gradual increase by slowing reabsorption. These are pharmacologically distinct enough that mirtazapine intolerance doesn't predict bupropion intolerance.
 - *What the evidence supports:* The OPTIMUM trial (2023, 619 adults aged 60+ with treatment-resistant depression) found that *adding* bupropion to an existing antidepressant produced about 28% remission — roughly equivalent to adding aripiprazole, and substantially better than *switching* antidepressants. So if bupropion is on the table, adding it to Cymbalta is the well-evidenced form. Switching from Cymbalta to bupropion was actually the worst-performing strategy in that trial.
 - *The biggest specific concern for Dad:* Bupropion augmentation had the highest fall rate of any arm in the OPTIMUM trial — about half a fall per patient over 10 weeks. With Dad's existing fall risk factors (gait and balance findings, tadalafil for BPH, slow heart rate, age over 70), this is the main caveat. Weekly check-ins for gait, balance, and blood-pressure-on-standing would be essential.
 - *Other relevant points:* Cardiac profile is favorable for Dad (doesn't slow heart rate; the modest blood pressure elevation actually helps move him toward normal from his low baseline of 112/58). Bupropion combined with ketamine is compatible — a 2022 paper specifically proposed that boosting dopamine-related activity (which bupropion does) may extend how long ketamine's antidepressant effect lasts.
 - *Family read:* A defensible third option if "do more than ketamine" pressure persists, and a substantially stronger evidence-based choice than Depakote. Sequencing: start the extended-release form at 150 mg about 2–3 weeks before the June 8 ketamine restart; hold at 150 mg for at least 4 weeks before any dose increase, given the interaction with Cymbalta. See new [handout #21](#).

WHERE THIS LANDS THE THREE OPERATIVE QUESTIONS

- **Cymbalta → Lexapro switch.** Dad has decades of good functioning on Lexapro at 10–20 mg, which is meaningful prior-responder data. The OE evidence against the switch is real but is largely about (a) Lexapro previously failed at *high doses* in 10/2025, (b) Del Casale's SNRI-favored signal, and (c) the discontinuation/activation window overlapping with the ketamine restart. The first concern is partly addressed by capping at 5–10 mg; the third is the real load-bearing problem. **Recommended position: defensible direction with sequencing caveats — run the first 2–3 ketamine sessions on the existing duloxetine background, then revisit the switch with response data in hand. If pre-ketamine timing isn't winnable, gradual cross-titration (1+ week of overlap at 30 mg duloxetine, then 20 mg, then off) and cap Lexapro at 5–10 mg.** See [handout #06](#).
- **Add Depakote.** No defensible indication; serious drug-induced parkinsonism risk in highest-risk Arnaldi profile; would confound the U-M Movement Disorders eval. **Recommended position: defer until indication is stated; if pursued, monitoring protocol per handout #01.** See [handout #01](#).
- **Remove trazodone + doxepin; add lemborexant.** Lemborexant alone (as an addition) is reasonable and Julane-endorsed; the doxepin → lemborexant *substitution* is seamless per OE #13. Removing trazodone-PRN simultaneously serves no purpose. **Recommended position: if Dad wants to come off doxepin, the swap to lemborexant 5 mg HS is defensible night-1; keep trazodone PRN.** See [handout #10](#).
- **Alternative augmentation (new live option): bupropion XL 150 mg.** If the conversation tomorrow lands on "we want to add something beyond ketamine and beyond the doxepin/lemborexant sleep adjustment," bupropion augmentation has stronger evidence than divalproex and a more defensible safety profile than Lexapro. Cautious trial; fall risk monitoring; sequenced 2–3 weeks ahead of the June 8 restart. **Recommended position: defensible third-line (behind ketamine + duloxetine continuation + behavioral); not first-line.** See new [handout #21](#).

WHAT WE STILL DON'T KNOW

- **Whether Chilton specifically "recommended" Depakote** or simply noted it was on the chart. Dad's memory is hesitant; Mom recalls Dad saying she recommended it. No written record.
- **What other recommendations Chilton may have made** that Dad didn't note.
- **Exactly what Dr. Rubin is recommending** — Rubin call tomorrow (5/20) at 11 AM.
- **What an A2 clinic intake looks like operationally** — protocol, room arrangement, in-room provider presence, pre-session anxiolysis approach.
- **Whether Dad is open to a single pre-session benzodiazepine dose** as part of the A2 intake.
- **Whether the post-session 5/18 coherence persisted or faded** over the next 24 hours.

FOLLOW-UPS

1. **Capture call notes after each provider conversation going forward.** Mom and/or Dad after each call.
2. **Tomorrow's 11 AM Rubin call:** the Cymbalta-switch and divalproex questions are now answerable from evidence. The conversation worth having is whether bupropion augmentation makes more sense than the current proposal as an alternative form of "doing more."
3. **Email Chilton:** confirm in writing whether divalproex was recommended or noted; ask for the evidence basis behind preferring Lexapro over Cymbalta as the ketamine background, given Del Casale 2025.
4. **Get Julane's written read** of yesterday's session and her sequencing preference.
5. **Confirm A2 clinic intake details:** date (June 8), protocol, in-room provider, pre-session anxiolysis approach.

RECOMMENDED PATH

1. **Push for sequencing on the Cymbalta → Lexapro switch:** ask Rubin to run the first 2–3 ketamine sessions on the existing duloxetine background, then revisit the switch with response data in hand. Preserves the documented Feb 2026 responder background through the highest-stakes window. If pre-ketamine timing isn't winnable, accept the switch but ask for a gradual cross-titration (≥ 1 week duloxetine 30 mg overlap, then 20 mg, then off) and a Lexapro dose cap at 5–10 mg

(the historical effective range, not the previously-failed high-dose range). Explicit symptoms that would trigger reversal (akathisia, persistent agitation, new motor symptom in first 2 weeks) defined up front.

2. **Defer divalproex** until an explicit indication is stated. If it proceeds despite that, full monitoring protocol per handout #01 with discontinuation triggers for any new motor or cognitive symptom.
3. **If doxepin comes off, swap to lemborexant 5 mg HS** on a single-night transition; keep trazodone 50 mg HS PRN; melatonin continues.
4. **If augmentation beyond ketamine is desired, bupropion XL 150 mg added to duloxetine is the strongest alternative to divalproex.** Start 2–3 weeks before June 8; hold at 150 mg $\geq$ 4 weeks before any uptitration; weekly check-ins with focused gait/balance and orthostatic monitoring.
5. **Weekly Julane sessions through the gap to June 8** — the strongest non-pharmacologic lever for the "felt alone" problem 5/18 surfaced. Use them for set/setting work, intention-setting, normalization of dissociation, and direct integration of last week's session.
6. **NPI top-tier items from [handout #19](#):** caffeine at 1 cup AM (already in place); bright-light parameters confirmed; digital CBT-I started; exercise to 3 $\times$ /week; alcohol abstinence. PSG with RBD montage prioritized in the U-M Movement Disorders coordination.
7. **Plan A2 intake with explicit pre-session anxiolysis** — single-dose lorazepam pre-session, in-room provider presence, set/setting continuity with Julane.

HOLDING ALONGSIDE THE CLINICAL *Dad has explicitly asked the family to maintain genuine hopefulness rather than false hope. Recovery from severe TRD of this duration is a long, slow climb, not a single intervention. Ketamine in the best of circumstances was the first step on a longer path, not a shortcut to the top. The 5/18 session was a hard step; it does not change the path or the destination. Continuing weekly Julane sessions through the gap is the form genuine hope takes in practice — the work is happening regardless of any infusion timing.*